

VETERINARY SERVICES FOR HOUSEHOLD PETS

Appendix N: Supporting evidence on prescription price controls

24 March 2026

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The Competition and Markets Authority has excluded from this published version of the final report information which the inquiry group considers should be excluded having regard to the three considerations set out in section 244 of the Enterprise Act 2002 (specified information: considerations relevant to disclosure). The omissions are indicated by [X]. Some numbers have been replaced by a range. These are shown in square brackets. Non-sensitive wording is also indicated in square brackets.

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1. Evidence on current and historical prescription fees in the market

- 1.1 This appendix sets out the evidence on current and historical prescription fees in the market which we have considered when determining the level of the prescription price control discussed in part B, section 5.¹
- 1.2 Due to the impracticality of setting a cap on prescription fees using a bottom-up cost analysis, we have considered the level of fees that veterinary businesses operating FOPs charge for written prescriptions in the market. We have gathered information on prescription pricing from a wide range of sources, including information on both current and historic fees going back to 2023. This shows that there has been a wide diversity of prescription fees over this period, with a notable difference between sites run by independent veterinary businesses and some of the LVGs.
- 1.3 We found that it is common for FOPs to charge several different types of prescription fee, which we have grouped into three categories and discuss in turn below:
- (a) **Primary prescription fees.** These are fees charged for the first medicine for which a written prescription is issued during a consultation.
 - (b) **Additional prescription fees.** These are fees which may be charged for any further medicines for which a written prescription is issued during a consultation (in addition to the medicine covered by the primary prescription fee).
 - (c) **Other prescription fees.** This category includes any other fees associated with written prescriptions.² For example, some FOPs may charge a different fee for repeat medications or offer discounted primary and additional prescription fees for charity or pet welfare reasons. These fees were significantly less common than primary and additional prescription fees and, therefore, have not materially influenced our analysis of the appropriate level for the price control.³

All prescription fee prices discussed in this appendix are inclusive of VAT.

¹ Part B, section 5: Medicines market opening remedies and prescription price control.

² We have only considered fees associated with written prescriptions, meaning prescriptions which a pet owner can use to purchase medicines at a source other than their FOP. Therefore, we have not considered other related fees such as dispensing or injection fees when determining the appropriate level for the Prescription Fee Caps.

³ Where relevant, we discuss below our treatment of these fees when explaining how we analysed each of the datasets on which we have relied.

Evidence on the market price of primary prescription fees

- 1.4 To inform the level of the price control on primary prescription fees, we gathered data from LVGs and independent FOPs on current and historical market prices. This subsection explains how we obtained and analysed each source of data and provides our view on its reliability and representativeness.

Data on primary prescription fees charged by LVG FOPs

- 1.5 We obtained data directly from all LVGs on their prescription pricing practices in 2023 and 2024, which were the most recent full datasets available. In most cases, this data was provided directly from PMS, providing us with highly reliable, granular, site level data on the fees charged by each LVG. Using this, we were able to accurately isolate primary prescription fees from all other types of fees at most LVGs.⁴
- 1.6 Almost all LVGs were able to provide data on a comprehensive proportion of their sites in both 2023 and 2024. One LVG [X] submitted that it could only provide detailed prescription pricing information for its sites which used the [X] Corporate PMS.⁵ It provided data on approximately 35% of its sites in 2023, and submitted that its external economic advisers [X] had reviewed the sample and confirmed it was representative of [X] small to medium sized practices, but that it was more limited for a small number of large multi-site practices.⁶ This LVG [X] provided data on a greater proportion of its sites (68%) for 2024.⁷
- 1.7 At many sites in this dataset, a very small number of prescriptions were issued at discounted prices on a charitable basis, creating multiple primary prescription fee price observations at each site. Additionally, a minority of sites contained other types of fees such as emergency prescription fees. To improve comparability across sites and avoid these fees materially impacting our results, we calculated a single average primary prescription fee for each site, weighted by the quantity of prescriptions sold at each price at a given site. The discounted fees and other types of fees represented approximately 3% of all primary prescriptions included in this analysis in both 2023 and 2024.⁸ We calculated average primary prescription fees for 2,175 LVG sites in 2023 and 2,416 in 2024.

⁴ Data provided by one LVG [X] did not have the same level of granularity as it does not categorise the type of prescription issued. LVG [X] response to RFI 21, Q2. We have therefore assumed that this data refers exclusively to primary prescription fees. Another LVG [X] submitted that although some FOPs use an alternative code to indicate additional prescription fees, some FOPs will use multiple entries of the same code that others typically use for primary prescription fees for prescribing additional medicines. Hence, we consider that some of the primary prescription fees for this LVG may actually be charges for additional medicines. LVG [X] response to RFI 21, Q3.

⁵ LVG [X] response to RFI 8, paragraph 2 and supporting bullets.

⁶ LVG [X] response to RFI 8, paragraph 2 and supporting bullets.

⁷ LVG [X] response to RFI 21, Q2

⁸ We have tested removing these fees in the submitted 2023 and 2024 LVG data and found that this did not change the median prescription fee.

Table 1.1: Primary prescription fees charged by LVG FOPs, 2023 (£)⁹

	Number of sites	Min	10 th percentile	Lower quartile	Median	Upper quartile	90 th percentile	Max
[X]	[X]	11.00	16.00	17.00	19.00	19.00	23.00	32.00
[X]	[X]	9.97	16.86	18.00	19.31	22.07	26.40	30.79
[X]	[X]	8.05	15.00	17.50	20.00	25.00	29.33	48.00
CVS	[X]	12.00	21.85	21.90	21.90	21.90	21.90	22.80
[X]	[X]	9.78	18.19	21.00	23.00	27.00	30.00	42.06
[X]	[X]	16.00	24.50	26.00	28.00	30.00	32.50	44.00
All LVG	2,175	8.05	17.33	20.00	22.97	27.53	30.50	48.00

Source: CMA analysis of data provided by LVGs (in response to RFI 8, Q1).

Notes: The figures reported for one LVG [X] differ from those in our PDR as [X] incorrectly submitted data exclusive of VAT. We have amended this data to be inclusive of VAT. LVG [X] response to RFI 22, Q1. Rows ordered by ascending LVG median prescription fee.

Table 1.2: Primary prescription fees charged by LVG FOPs, 2024 (£)

	Number of sites	Min	10 th percentile	Lower quartile	Median	Upper quartile	90 th percentile	Max
[X]	[X]	10.00	16.00	18.50	20.00	23.00	25.14	42.50
[X]	[X]	10.79	18.50	20.00	21.16	25.00	26.80	38.50
[X]	[X]	14.00	22.00	22.00	24.00	24.00	26.00	31.00
CVS	[X]	21.49	24.62	24.70	24.70	24.70	24.70	24.70
[X]	[X]	11.54	20.00	21.91	24.83	28.08	30.00	45.64
[X]	[X]	16.50	25.98	28.50	30.00	32.60	35.00	45.00
All LVG	2,416	10.00	19.99	22.50	24.70	29.50	32.50	45.64

Source: CMA analysis of data provided by LVGs (in response to RFI 21, Q2).

Notes: Rows ordered by ascending LVG median prescription fee.

- 1.8 As shown in Tables 1.1 and 1.2 above,¹⁰ in both 2023 and 2024, five of the LVGs (CVS, [X], [X], [X] and [X]) had similar median prices for primary prescription fees, with a price difference of less than £5.00 between them in each year. The sixth LVG [X] had higher prices than the other five in both 2023 and 2024.
- 1.9 Most LVGs charged a range of prices across practices, with differences of between £20 and £30 between the lowest and highest prices charged. However, CVS sets prescription prices nationally so, with the exception of a few practices, had consistent primary prescription fees across practices.¹¹
- 1.10 The median prescription fee charged across all LVGs was £22.97 in 2023 and £24.70 in 2024. Median primary prescription fees for four LVGs (CVS, [X], [X], and [X]) were approximately 10% higher in 2024 than in 2023. The median primary prescription fee was unchanged for another LVG [X]. One LVG's [X] median primary prescription fee was 26% higher in 2024 than in 2023. This LVG charged £19.00 in 2023 and £24.00 in 2024.¹² While this price rise is larger than

⁹ Please note that each of these tables are ordered by the size of the median fee in 2024.

¹⁰ Some LVGs [X], [X] and [X] carried out analysis to replicate our figures using their data that they submitted to us. These LVGs reached estimates that were very similar to what we produced (within 50 pence). Given the CMA's methodology was adapted to account for differences in the data provided by different LVGs, we would not expect a separate analysis to produce an identical figure.

¹¹ CVS response to RFI 21, Q1. CVS response to RFI 1, Q10.

¹² We note that, as the data provided by two LVGs [X] and [X] did not cover a comprehensive number of their sites in both 2023 and 2024, different sites may have been included in the sample for each year. Therefore, differences in median prices at these LVGs between 2023 and 2024 may not be truly reflective of broad-based price increases at these LVGs' FOPs. This potential limitation does not affect our ability to compare these LVGs to others in the same year.

those reported by other LVGs, this may be at least partially explained by a timing mismatch in the data provided by this LVG compared to others.¹³

Data on primary prescription fees charged by independent practices

- 1.11 Although LVG-owned FOPs account for a majority of the market, it is important to take account of what fees independent vets charge. However, obtaining data on the prescription fees charged by independent FOPs is naturally more challenging than gathering equivalent data on the LVGs as there is no single source which covers all independent FOPs.
- 1.12 In the PDR, we primarily used one dataset to understand the pricing of prescription fees across the independent sector. The median prescription fee in this dataset was £15.¹⁴ Following the response from the sector that this data, and the proposed level of the price cap, was not consistent with the pricing of independent FOPs, we carried out extensive additional data collection and quality checks on our existing dataset. This process identified errors in the information provided to the CMA, which have been corrected for in our analysis. We set out below our evaluation of each dataset that we have collected to help understand prescription fee pricing in the independent sector, and their strengths and weaknesses.
- 1.13 Overall, the datasets indicate that independent FOPs charge a wide range of fees but are on average less expensive than LVG-owned FOPs. We have reviewed a substantial amount of information on prices in 2024. However, these datasets contain less information for 2023 and provide only limited more recent data for 2025-2026. Across our datasets, median independent primary prescription fees ranged from £18.00 to £21.55 in 2023, £18.44 to £24.00 in 2024, and £22.00 to £25.00 in 2025-26.

Data from veterinary advisory businesses [redacted] [redacted]

- 1.14 We used data from two veterinary advisory businesses [redacted] [redacted] to assess the price of prescription fees charged by independent vets.

First veterinary advisory business [redacted]

- 1.15 The dataset received from this veterinary advisory business [redacted] receives data drawn directly from its customers' PMS, which it processes to provide business

¹³ This LVG [redacted] provided 2023 price information from January 2023, while other LVGs provided data from December 2023. The LVG [redacted] submitted that it increased prices in May 2023, which would not be captured by this data. All LVGs provided data from the final quarter of 2024. LVG [redacted] response to RFI 22, Q1. All LVG responses to RFI 8, Q1; All LVG responses to RFI 21, Q2.

¹⁴ This fee was inclusive of all types of prescription fee as we did not separate primary and additional fees in our PDR. More details are available in our [Provisional decision report: part B](#), paragraphs 6.18 to 6.22.

strategy advice.¹⁵ This is the dataset that underpinned our analysis in the PDR. However, following quality control checks against the additional data sources we collected post-PDR, we identified that the data had incorrectly been provided on the basis that it included VAT when, in fact, it excluded VAT.¹⁶ This is part of the reason that the fees in this dataset in our analysis for the PDR were substantially lower than most of the LVGs' fees. We have relied on the corrected data in our analysis for this Final Report.

- 1.16 To identify primary prescription fees, we filtered the data for treatments described as prescriptions, removing any refunds and observations with negative or zero quantities. We categorised prescriptions which did not contain terms such as 'additional' or '2nd', and those described as 'per item', to be primary prescription fees. We removed FOPs described as 'equine' or 'farm' from the dataset to keep only small animal FOPs. Due to the size of the dataset, we were unable to remove the small number of non-primary prescription fees not captured by the data cleaning described above. This may include a small number of discounted or charity prescription fees, and other types of fees such as those for emergency prescriptions.
- 1.17 About 3% of observations had a reported quantity of less than or more than one where quantity represented the number of prescriptions or dosage of medicine prescribed.¹⁷ We standardise prices by dividing by reported quantity before calculating a single average primary prescription fee for each site. This resulted in a dataset containing:
- (a) 143 sites (owned by 65 veterinary businesses) covering prescriptions charged throughout 2023; and
 - (b) 125 sites (owned by 56 veterinary businesses) covering prescriptions charged between January 2024 and June 2024.

Table 1.3: Primary prescription fees charged by customers of a veterinary advisory business, 2023 and 2024 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	90 th percentile	Max
2023 results	143	0.00	14.33	18.00	22.00	25.78	46.06
2024 results	125	9.00	14.33	18.44	22.64	25.66	31.93

Source: CMA analysis of data provided by a veterinary advisory business [redacted]. Vet advisory business [redacted] response to RFI 1, and vet advisory business [redacted] response to RFI2, Q1.

¹⁵ Veterinary advisory business [redacted] response to CMA follow up questions, September 2024.

¹⁶ Veterinary advisory business [redacted] response to follow up to RFI 2, Q1.

¹⁷ Veterinary advisory business [redacted] response to CMA follow up questions, September 2024.

Note: Figures for 2023 differ from those reported in our PDR because we have: (i) adjusted prices which were incorrectly reported as multiples of their single item value; (ii) added VAT; and (iii) removed an outlier (the maximum reported in the PDR) as it appears this datapoint covered the combined cost of a consultation and written prescription.

- 1.18 As shown in table 1.3 above, the median primary prescription fee charged by independent FOPs in this dataset was £18.00 in 2023 and £18.44 in 2024. In both years, more than 90% of independent FOPs in this dataset had primary prescription fees under £26.00, and few had primary prescription fees above £30.00.
- 1.19 Despite this dataset covering a relatively limited sample of independent vets, we consider it to be the highest quality dataset covering independent vets of the data we have collected. This is because it records data directly from PMS, and also contains detailed prescription information allowing us to isolate primary and additional fees at sites that offer both and remove other irrelevant fees. However, the veterinary advisory business submitted that its customers have ‘higher than average financial performance’,¹⁸ which we consider may make the data less representative of less financially secure independent FOPs. Therefore, while this data accurately reflects the fees charged by customers of the veterinary advisory business, it may not be representative of the independent market overall.

Second veterinary advisory business [X]

- 1.20 We also gathered data from a second veterinary advisory business [X]. This business told us that it held data on 42 independent FOPs based on a survey carried out in May 2025.¹⁹ The median primary prescription fee at these businesses was £20.00. However, we consider that this dataset is highly unlikely to be representative of all independent FOPs due to the small sample size (approximately three times smaller than the other veterinary advisory business dataset discussed above).

Data from a buying group [X]

- 1.21 We gathered data from the members of a buying group²⁰ consisting of [X] veterinary businesses operating 113 independent FOPs. By seeking data from the existing members of a buying group, we sought to avoid issues of selection and response bias as the sample was limited to a clearly defined group of practices. Moreover, the limited number allowed us to obtain the data through a survey emailed directly to buying group members and we received a 100% response rate.

¹⁸ Veterinary advisory business [X] response to RFI 2, Q3.

¹⁹ Veterinary advisory business [X] email correspondence, 15 January 2026.; Veterinary advisory business [X] email correspondence, 12 February 2026

²⁰ Buying groups, including the role they play in the supply of veterinary medicines, are explained in part A, section 11: Veterinary Medicines, sub-section Buying groups enable FOPs and third-party retailers to obtain larger rebates.

Most members of this buying group provided both current and historical price information.²¹

Table 1.4: Primary prescription fees charged by a buying group's members, 2023, 2024, and 2026 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	90 th percentile	Max
2023 results	[3<]	13.86	17.50	21.50	23.14	27.70	30.00
2024 results	[3<]	16.50	20.00	24.00	25.00	30.09	35.00
2026 results	[3<]	16.00	20.00	25.00	26.48	31.69	37.00

Source: CMA calculations based on results obtained from [3<] buying group members, January 2026.

- 1.22 As shown in Table 1.4 above, the median primary prescription fee charged by the buying group members has risen from £21.50 in 2023 to £24.00 in 2024, and to £25.00 in 2026. Members of the buying group charged a wide range of primary prescription fees, with the highest fee being more than double the lowest fee in any given year.
- 1.23 Members of this buying group may not be representative of all independent FOPs. The buying group submitted that its members are concentrated in rural areas and are less present in the south-east of England than FOPs generally.²² It submitted that many of its members originated as mixed farm and small animal practices and that this heritage informs their professional ethos and may influence pricing strategies.²³ The buying group noted the sustainable level of profitability of its members, though it could not indicate whether this sustainability would mean higher or lower prices relative to the average independent FOP.²⁴ Considering these factors, and the relatively limited sample size of buying group members, our view is that this dataset represents independent FOPs with specific characteristics which may make them unrepresentative of the wider independent sector.

Data drawn from website reviews

- 1.24 In order to obtain data on as wide a range of independent FOPs as possible, we have gathered data from price comparison websites (**PCWs**) and conducted our own review of primary prescription prices available on FOP websites.
- 1.25 While these data sources allow us to evaluate the prices of a far greater number of FOPs than through our direct survey, any dataset composed of prices drawn primarily from websites will suffer from publication bias. As FOPs can currently choose whether to display price information on their websites, the sample of FOPs which provide this information may not be representative of the population. This effect could either bias published prices higher (if FOPs who are more

²¹ We obtained 2024 price data for [3<] sites and 2023 price data for [3<] sites out of [3<] total sites.

²² Submission from a buying group [3<], January 2026, p.1

²³ Submission from a buying group [3<], January 2026, p.1

²⁴ Submission from a buying group [3<], January 2026, p.2

commercially minded and therefore charging higher prices choose to publish price lists more often) or lower (if the most competitively priced FOPs are more likely to choose to promote their prices). In addition to publication bias, the reliability of website results can be impacted by FOPs or PCWs which do not maintain up-to-date price and ownership information.

1.26 We used data from a PCW [redacted] to test for publication bias. We compared data held by this provider on LVG primary prescription fees to the comprehensive LVG data discussed above. While most LVGs' published prices were slightly higher than in the comprehensive LVG dataset,²⁵ we do not consider that this is clear evidence of publication bias as the PCW data is more up-to-date and therefore it is possible that price differences are caused by this timing mismatch rather than solely publication bias.²⁶

Vet Help Direct

1.27 We obtained data from Vet Help Direct, a large PCW which held data on the primary prescription fees charged by 656 independent sites in 2024. This is the single largest source of data on independent prescription fees that we have reviewed.

Table 1.5: Primary prescription fees on independent FOP websites reviewed by Vet Help Direct, 2024 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	90 th percentile	Max
2024 results	656	9.95	18.00	20.00	25.00	27.30	52.20

Source: CMA calculations based on Vet Help Direct submission dated January 2026.

1.28 As shown in Table 1.5 above, the median primary prescription fee charged in Vet Help Direct's sample was £20.00 in 2024. The lowest priced independent in the dataset charged less than £10.00, and more than 90% of independent FOPs in this sample charged less than £28.00.

1.29 Vet Help Direct told us that it collects information primarily by contacting FOP practices directly, although it also scrapes data from websites in some cases.²⁷ Vet Help Direct was unable to provide information on current or 2023 prices for a majority of the sites it holds data for as it stopped updating its price lists at the end

²⁵ LVG 1's [redacted] median published price was £2.30 higher than the median of the data it submitted to us, LVG 2's [redacted] published median was £5.00 higher, LVG 3's [redacted] published median was £2.16 lower, LVG 4's [redacted] published median was £2.00 higher, and LVG 5's [redacted] published median was £1.02 higher. The PCW was unable to provide any published primary prescription fee prices for Medivet FOPs. CMA calculations based on PCW [redacted] response to RFI.

²⁶ This PCW's data comes from October 2025 to January 2026. The LVGs provided data covering the fourth quarter of 2024.

²⁷ For example, Vet Help Direct email correspondence, January 2026; Note of a call with Vet Help Direct, January 2026, p.1; and Vet Help Direct response to putback.

of 2024.²⁸ Prescription fees were collected at multiple points throughout 2024 rather than on a single date.

- 1.30 We excluded from our analysis observations from FOPs with missing data on either the data collection date or the variable indicating they were independently owned. We could not verify that these observations represented independently owned FOPs' prescription fees from 2024. We note that these exclusions did not change the median, lower quartile or upper quartile results.
- 1.31 We consider that this dataset is likely to be significantly more reliable and representative than our other website-based datasets. As mentioned above, this dataset covers a relatively large sample of independent FOPs²⁹ – larger than any other source available to us – and may be less impacted by publication bias as it includes sites which have been contacted directly by Vet Help Direct.

Second PCW [redacted]

- 1.32 We obtained price information from a second, smaller, PCW [redacted]. This dataset included primary prescription price information on 397 sites, 59 of which were independent FOPs. This data was gathered from practice websites between October 2025 and January 2026.³⁰
- 1.33 The data provided by the PCW included information on several types of prescriptions. We isolated primary prescription fees by manually categorising each fee in the dataset and removing fees which included terms such as 'additional', '2nd' or 'repeat'. The final sample included 56 unique independent sites.³¹

Table 1.6: Primary prescription fees on independent FOP websites reviewed by second PCW, 2026 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	90 th percentile	Max
2025-2026 Results	56*	13.36	20.00	22.95	25.00	29.17	35.00

Source: CMA calculations based on [redacted] response to RFI.

* One site was included twice with two different primary fees, one for general medicines and another for controlled drugs. Both fees have been included meaning the analysis includes 57 total datapoints.

²⁸ Vet Help Direct told us that it has continued to update price information for FOPs which request this. However, it told us that this represents a relatively small proportion of the FOPs in this dataset. In any case, we have excluded these updated prices from our 2024 analysis. Note of call with Vet Help Direct, January 2026, p.1

²⁹ We estimate there are at least 1,767 independent FOPs in the market. Therefore, Vet Help Direct's dataset covers approximately 37% of the independent market. More details on the number of independent FOPs in the market are available in part A, section 2: context for the veterinary services market.

³⁰ Price comparison website [redacted] submission, 22 January 2026.

³¹ One site was included twice with two different primary fees, one for general medicines and another for controlled drugs. Both fees have been retained meaning the analysis includes 57 total datapoints.

- 1.34 The median primary prescription fee charged by independent FOPs in the dataset provided by this PCW was £22.95. The lowest priced independent in this dataset charged £13.36, and 90% of respondents charged £29.17 or less.
- 1.35 However, due to the very small number of independent FOPs in this dataset and the potential for publication bias,³² we do not consider that this dataset provides a reliable indication of the primary prescription fee prices charged by all independent FOPs.

Our website review

- 1.36 In addition to the data provided by third-party PCWs, we conducted a review of a sample of websites which we had previously identified as having prescription pricing information. We reviewed an initial sample of 462 independent sites,³³ and found 122 sites contained accessible information on primary prescription fees.³⁴

Table 1.7: Primary prescription fees on independent FOP websites reviewed by us, 2026 (£)

	<i>Number of sites</i>	<i>Min</i>	<i>Lower quartile</i>	<i>Median</i>	<i>Upper quartile</i>	<i>90th percentile</i>	<i>Max</i>
2026 results	122	7.20	16.00	20.00	26.48	28.11	33.65

Source: CMA website review, January-March 2026.

- 1.37 The median primary prescription fee charged by the sample of independent FOPs we reviewed was £20.00. The lowest priced independent in this dataset charged £7.20, and 90% of respondents charged £28.11 or less.
- 1.38 As only 20% of the independent FOP websites we reviewed contained information on primary prescription fees, it is likely that this small sample is significantly affected by publication bias. Therefore, we do not consider this sample to be representative of all independent FOPs.

Results of the SPVS fee survey

- 1.39 We also obtained the underlying data used by SPVS to produce its 2023 and 2024 fee survey reports. Many respondents to our PDR submitted that we should take

³² We note that, unlike the data provided by Vet Help Direct, this PCW has not gathered additional price information directly from FOPs through phone calls and emails. As a result, this dataset is more likely to be affected by publication bias than Vet Help Direct.

³³ More details of our website review are available in part A, section 8: competition between FOPs, subsection CMA research of online pricing information.

³⁴ We did an initial review of all independent sites in our sample in January 2026, and in early 2026 we re-reviewed the medium sized independent sites to ensure that our analysis was as up to date as possible, while avoiding a re-review of all sites. In early 2026 we reviewed all sites that had been identified as having accessible prescription pricing, to extract the detailed prescription pricing information that we required for our analysis.

account of the results of the SPVS fee survey when setting the prescription price cap.³⁵

1.40 SPVS submitted data collected from 144 FOPs in 2023 and 117 in 2024.³⁶ Of these, 137 included data on prescription fees for small animals in 2023, and 115 did so in 2024. After cleaning the data to remove results that appeared to be entered in error, we obtained a final sample of 135 sites in 2023 and 114 in 2024.³⁷ We note that the SPVS fee survey does not ask respondents to provide information specifically about, or distinguish between, primary or additional prescription fees.³⁸ Therefore, we have assumed that all responses relate to the primary prescription fee charged by these sites.

Table 1.8: SPVS fee survey results in 2023 and 2024 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	90 th percentile	Max
2023 results	135	10.08	18.50	21.55	25.00	29.54	36.60
2024 results	114	10.80	19.26	22.50	26.47	30.00	39.60

Source: CMA calculations based on SPVS response to draft RFI, Q2.

1.41 The median fee charged by participants in the SPVS fee survey in 2023 was £21.55 in 2023 and £22.50 in 2024. As observed in other sources, there is a wide range of prescription fees charged by FOPs surveyed by SPVS, with the highest fee in each year being three to four times greater than the lowest fee. In both years, 90% of respondents to the SPVS fee survey charged £30.00 or less for a written prescription.

1.42 There are a number of limitations to the reliability and robustness of the SPVS data. In particular:

- (a) The survey is not a survey of independent practices and does not provide clear data on pricing of prescription fees at independent FOPs. SPVS did not collect any identifying information about the FOPs which responded to its survey, including whether they are SPVS members. As a result, we have not been able to assess the representativeness of its sample or whether its respondents are independent or part of an LVG. SPVS submitted that 70% of its members are independent FOPs and so it expects that the majority of responses, though not all, would come from independent practices.³⁹ Given

³⁵ For example: BVA BVNA BSAVA SPVS VMG p 22; Briar Vets Ltd p4; Scott Veterinary Clinic p4; VetPartners p 57.

³⁶ As discussed below, SPVS did not collect information on whether these sites are operated by LVGs or independent vets.

³⁷ We removed sites which reported charging £0 for a written prescription. This did not materially affect the results.

³⁸ In both 2023 and 2024, the survey asked participants: ‘How much do you charge to issue a prescription to a small animal client?’ SPVS response to RFI, Q2.

³⁹ SPVS response to RFI, Q3. As discussed below, a link to the survey was made publicly available, meaning FOPs did not need to be members of SPVS to complete the survey. Therefore, it is possible that SPVS’ membership ratio is not a good approximation for the ratio of independent and LVG respondents to the survey.

the evidence above showing that LVGs on average have higher primary prescription fees than independent FOPs in most of our sources, we consider that the presence of LVG respondents would increase the median price reported in the survey compared to a sample in which we were able to remove these LVG FOPs and retain only independent responses.⁴⁰

- (b) The survey itself has methodological challenges which means there are significant risks that the data is not robust. For example, a link to respond to the survey was made publicly available on SPVS' website. As it is a long survey that does not collect identifying information and only survey respondents gain access to the results,⁴¹ some practices may be incentivised to provide approximate or best-guess price information to gain access to the results. Given the nature of the data provided, it is not possible for us to carry out meaningful quality assurance checks, nor did SPVS indicate that it performs quality assurance checks on the responses.⁴² SPVS submitted that the survey relied on trust, respondents had no reason to lie, and that the results were likely to be truthful as the survey results recorded similar patterns each year.⁴³

- 1.43 Overall, while this dataset is of a similar size to the datasets provided by the veterinary advisory business and buying group described above, we consider that it is less informative of the prescription fees charged by independent FOPs as it contains an unknown proportion of LVG sites. Based on SPVS membership, it is possible that approximately 30% of responses could come from LVG-owned FOPs, potentially biasing the median reported by the survey above the true level of independent FOPs.

Cross-referencing of data sources, and estimate of the total number of Independent FOPs reviewed

- 1.44 Our analysis of independent FOPs' prescription fees comes from a range of sources, which differ in how much detail is provided about the practices. There are also data sources included in our analysis which do not include details on the practice site – primarily the SPVS dataset, which is one of the larger datasets, but does not include any information on the site or business.

⁴⁰ We tested the impact of adding LVG sites to an independent dataset using the data provided by one of the veterinary advisory businesses in 2024 as discussed above. We weighted the sample to contain 30% LVG FOPs and 70% independent FOPs. Adding these LVG FOPs caused the median primary prescription fee to rise 10%, from £18.44 in the original veterinary advisory business dataset to £20.31 in our mixed sample. We note that this level is only around £2 lower than the median observed in the SPVS dataset. This is consistent with our assessment that the presence of LVG FOPs in the SPVS dataset may bias reported primary prescription fee prices higher than in a sample consisting entirely of independent FOPs.

⁴¹ SPVS response to RFI, Q3.

⁴² We asked SPVS to provide details on any quality assurance or controls it had undertaken. In its response, it did not describe any such checks. SPVS response to draft RFI, Q3

⁴³ SPVS response to draft RFI, Q3.

- 1.45 However, from the datasets that did include practice names, we were able to estimate the number of unique sites that were unique in our analysis and those sites which were duplicated between two or more of our datasets.
- 1.46 We compared the lists of practice names and found 912 individual FOP sites were included across all the samples.⁴⁴ We can therefore be confident that more than 900 independent FOPs were covered by our analysis. There were at least 1,767 independent FOPs active in the market in 2025,⁴⁵ so our coverage is approaching half of the independent sector.⁴⁶ Given the practical limitations in gathering data from independent FOPs in a way that is proportionate and not unduly burdensome for independents, it was not possible to gather a representative or random sample. However, this strong coverage from a range of sources gives us greater confidence in our understanding of the level of prescription fees set by independent FOPs.
- 1.47 We identified 122 FOP sites that were duplicated in at least two of our data sets. Where possible, we used these overlapping sites to carry out cross checks on the accuracy of the data. Some of the datasets were from different time periods and, therefore, not directly comparable – but this analysis allowed us to check for errors or any systematic bias between the datasets. These quality checks identified the erroneous omission of VAT from one of our datasets (as discussed above).

Evidence on the market price of additional prescription fees

- 1.48 To inform the level of the price control on additional prescription fees, we gathered data from LVGs and independent FOPs on current and historical market prices. While we have observed that it is common practice for FOPs to offer both primary and additional prescription fees, not all FOPs choose to structure their prescription pricing in this way. As a result, the information we hold on additional prescription fees is less comprehensive than for primary prescription fees. We note the extent of this limitation when discussing each dataset below.
- 1.49 In addition to analysing the prices charged for additional prescription fees, we have considered how these prices compare to the median fee charged for a primary prescription within the same dataset. As discussed in Part B Section 5, Prescription price cap remedy, we consider that the relatively stable price differences between primary and additional fees we observe within most sources

⁴⁴ We did not have sufficient information to check other information on practices, for example, only one of our datasets included post codes.

⁴⁵ This figure represents independent FOPs which we have 'confirmed' exist and is therefore likely to be an underestimate of the total number of independent FOPs. More details are available in part A, section 2: context for the veterinary services market.

⁴⁶ As noted above, coverage is not consistent between years. We hold the largest amount of data for 2024, covering almost half of the independent market, compared to around 12% in 2023 and around 10% in 2025-26.

over time are informative evidence for setting the level of the Additional Prescription Fee Cap.

Data on additional prescription fees charged by LVG practices

- 1.50 From the data provided by LVGs, we are only able to identify additional prescription fee charges in the data submitted by four LVGs [3<], [3<], [3<] and [3<]. Of the remaining LVGs, CVS submitted that its primary prescription fee covers up to five medicines for a single condition,⁴⁷ and so it does not separately categorise prescription fees for additional medicines. In effect, we consider this to mean that it has an additional prescription fee of £0.00, as we assume that instances where a second prescription is charged would be rare. The other LVG [3<] submitted that it was unable to provide any details of the type of prescriptions issued by its FOPs as it only centrally holds data confirming that a prescription was charged at a specific FOP, and does not collect data on the medicines prescribed or the context.⁴⁸ Hence, we were unable to identify whether FOPs owned by this LVG [3<] charge additional prescription fees.
- 1.51 As discussed above, in instances where there were multiple prices charged for additional prescriptions at the same site, we calculated a single average additional prescription fee for each site, weighted by the quantity of prescriptions sold at each price at a given site. We did not find any instances where fees were offered at charitable discounts for additional prescriptions in both 2023 and 2024. We calculated average additional prescription fees for 828 LVG sites in 2023 and 963 in 2024.

Table 1.9: Additional prescription fees charged by FOPs, 2023 (£)

	Number of sites	Min	10 th percentile	Lower quartile	Median	Upper quartile	90 th percentile	Max	Median as % of median primary fee
CVS*	[3<]	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0%
[3<]	[3<]	6.00	9.00	9.00	10.00	10.00	13.00	18.00	53%
[3<]	[3<]	2.63	4.00	6.04	12.50	17.00	20.00	30.79	65%
[3<]	[3<]	7.50	7.74	12.41	15.00	17.00	22.50	46.27	65%
[3<]	[3<]	6.40	15.00	18.00	20.54	24.61	30.00	64.20	73%
All four LVGs†	828	2.63	8.00	10.00	14.75	20.00	25.00	64.20	57%

Source: CMA analysis of data provided by LVGs (in response to RFI8, Q1).

* For CVS, an additional prescription fee of £0.00 is assumed as the primary prescription fee covers the cost of up to five medicines for a single condition.

† Not including CVS where we have assumed an additional prescription fee of £0.00.

Notes: Rows are ordered by ascending LVG median fees.

⁴⁷ CVS further submitted that, in practice, where a pet presents with more than one condition requiring medication, it is common for its vets to charge a single prescription fee for multiple medicines even when these are for different conditions. CVS response to RFI 21, Q1(b).

⁴⁸ LVG [3<] response to RFI 21, Q2.

Table 1.10: Additional prescription fees charged by FOPs, 2024 (£)

	Number of sites	Min	10 th percentile	Lower quartile	Median	Upper quartile	90 th percentile	Max	Median as % of median primary fee
CVS*	[X]	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0%
[X]	[X]	8.00	12.00	12.00	13.00	13.00	14.00	17.00	54%
[X]	[X]	3.08	4.06	7.00	13.96	18.52	21.53	24.78	66%
[X]	[X]	7.50	10.00	13.56	16.50	21.12	24.60	47.99	66%
[X]	[X]	11.50	16.63	20.00	22.50	25.31	35.00	65.00	75%
All four LVGs†	963	3.08	11.50	13.00	15.00	22.00	25.50	65.00	55%

Source: CMA analysis of data provided by LVGs (in response to RFI21, Q2).

* For CVS, an additional prescription fee of £0.00 is assumed as the primary prescription fee covers the cost of up to five medicines for a single condition.

† Not including CVS where we have assumed an additional prescription fee of £0.00.

Notes: Rows are ordered by ascending LVG median fees.

- 1.52 The median additional prescription fee charged across the four LVGs which price separately additional prescription fees and for which we have data was £14.75 in 2023 and £15.00 in 2024. In 2023, the median additional prescription fee charged by each LVG varied significantly, as the most expensive LVG [X] charged £20.54, more than double the fee of the least expensive LVG [X], which charged only £10.00. In 2024, there was less differentiation between the LVGs. The median additional prescription fee charged by [X] remained the lowest (£13.00), closely followed by [X] (£13.96). [X] priced higher, charging a median fee of £16.50, and [X] remained the most expensive, with a median price of £22.50.
- 1.53 Tables 1.9 and 1.10⁴⁹ show a very wide range of additional prescription fee prices charged by LVGs in our dataset. However, when looking at the median additional prescription fee as a proportion of the median primary prescription fee charged by a given LVG, we observe less variation. Across all four LVGs for which we have data on additional prescription fees, the median additional prescription fee was 57% of the median primary prescription fee charged by these LVGs in 2023. [X] median additional fee was the lowest in comparison to its median primary fee (53%), while [X] median additional fee was closest to its median primary fee (73%). In between, both [X] and [X] median additional prescription fees were 65% of their median primary prescription fees in 2023. All of these proportions remained stable in 2024.⁵⁰

⁴⁹ An LVG [X] carried out analysis to replicate our figures using their data that they submitted to us. This LVG reached estimates that were very similar to what we produced (within 50 pence). Given the CMA's methodology was adapted to account for differences in the data provided by different LVGs, we would not expect a separate analysis to produce an identical figure.

⁵⁰ The proportion across all four LVGs for which we have data fell by 1 percentage point (pp) to 55% in 2024. The proportions for each individual LVG changed from 2023 to 2024 as follows: [X] increased 2pp to 75%; [X] increased 1pp to 54%; [X] increased by 1pp to 66%; and [X] increased by 1pp to 66%.

Data on additional prescription fees charged by independent practices

- 1.54 The data that we have reviewed on additional prescription fees charged by independent FOPs is primarily from the datasets provided by one of the veterinary advisory businesses and the members of a buying group, covering prices in 2023 and 2024. For 2025/2026, we analysed data on additional prescription fees from members of a buying group, one of the PCWs, and our website review.
- 1.55 However, the data we have gathered on additional prescription fees charged by independent FOPs is less comprehensive than for primary prescription fees. This is because not all independent FOPs choose to charge a different fee for primary and additional prescriptions, not all FOPs offering additional prescription fees will choose to publish this information, and not all data aggregators, such as PCWs, will collect this information. As a result, our largest data source for primary prescription fees, Vet Help Direct, did not provide any information on additional prescription fees. The PCW and web review datasets also have small sample sizes and so are significantly less robust than the veterinary advisory business and buying group datasets.
- 1.56 The available data shows that many, but not all, independent FOPs price additional prescription fees significantly lower than their primary prescription fee. Across our datasets, median independent primary prescription fees ranged from £6.92 to £17.90 in 2023, £8.00 to £20.00 in 2024, and £11.00 to £22.00 in 2025-26. Across all years, median additional fees as a proportion of primary fees varied widely between our sources from 38% to 88%, reflecting heterogeneity in the approaches to pricing taken in the independent sector. However, within each dataset, this proportion remained relatively consistent between years – consistent with the evidence discussed above on LVG pricing. This proportion did not vary by more than approximately 5% between years in any of our independent FOP datasets. This consistency may indicate that many FOPs set the price of their additional prescription fee as a proportion of their primary prescription fee.

Data from a veterinary advisory business [X]

- 1.57 Using the approach described in the previous ‘data from veterinary advisory businesses’ subsection, we isolated additional prescription fees in the data provided by one veterinary advisory business [X].
- 1.58 We identified additional prescription fees as prescriptions containing terms such as ‘additional’ or ‘2nd’ and prescriptions described as ‘per item’ fees. Some FOPs in this dataset may not separately track primary and additional prescription fees. In these cases, we have treated all that site’s fees as primary prescription fees and so they are excluded from our analysis of additional prescription fees. Following our previous approach, we standardised prices by dividing by the reported quantity

and calculated a single average additional prescription fee for each site.⁵¹ This resulted in a dataset containing 48 sites (owned by 14 veterinary businesses) covering prescriptions charged throughout 2023, and 52 sites (owned by 14 veterinary businesses) covering prescriptions charged between January 2024 and June 2024.

Table 1.11: Additional prescription fees charged by customers of a veterinary advisory business, 2023 and 2024 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	Max	Median as % of median primary fee
2023 results	48	5.51	5.51	6.92	10.86	32.45	38%
2024 results	52	0.00	7.19	8.00	11.57	17.58	43%

Source: CMA analysis of data provided by a veterinary advisory business [X]. [X] response to RF11. [X] response to RF12. .

- 1.59 The median additional prescription fee charged by customers of a veterinary advisory business was £6.92 in 2023 and £8.00 in 2024. Other than one outlier which charged £32, there was limited variation in the prices charged by the most and least expensive 25% of sites.⁵² The median additional prescription fee charged by independent FOPs in 2023 was 38% of the median primary prescription fee. In 2024, this proportion was 43%.

Data from a buying group [X]

- 1.60 We obtained data from [X] veterinary businesses operating 113 FOPs that are members of a buying group [X]. All 113 FOPs provided information on their current additional prescription fees and most provided data on their historical prices in 2023 and 2024.⁵³

Table 1.12: Additional prescription fees charged by a buying group's members, 2023, 2024, and 2026 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	Max	Median as % of median primary fee
2023 results	[X]	0.00	13.86	17.90	22.56	30.00	83%
2024 results	[X]	0.00	15.72	20.00	25.00	34.00	83%
2026 results	[X]	0.00	13.24	22.00	26.36	41.69	88%

Source: CMA calculations based on results obtained from [X] buying group members, January 2026.

- 1.61 As shown in Table 1.12 above, the median additional prescription fee charged by the buying group's members rose from £17.90 in 2023, to £20.00 in 2024 and to £22.00 in 2026. In all years, more than a quarter of the buying group's members charged less than £16 for an additional prescription. Three veterinary businesses,

⁵¹ Due to the large number of observations in the dataset, we were unable to remove the small number of non-additional prescription fees not captured by the data cleaning described above. This may include a small number of discounted or charity prescription fees, and other types of fees such as those for emergency prescriptions.

⁵² Measured as the difference between the upper and lower quartiles in Table 1.11.

⁵³ [X] sites held data on their additional prescription fee charge in 2024. [X] sites held information on their 2023 prices.

operating nine FOPs, did not charge an additional prescription fee in 2026.⁵⁴ The median additional prescription fee charged by the buying group’s members in 2023 was 83% of the median primary prescription fee. This proportion was stable in 2024 (83%) but rose to 88% in 2026.

Data drawn from website reviews

1.62 Very little information on additional prescription fees was available in the datasets gathered from FOP websites, including both data from PCWs and our website review. Our largest PCW dataset, provided by Vet Help Direct, did not include any information on additional prescription fees. Of the 59 independent FOPs included in the second PCW’s [3<] dataset, only 16 included information on additional prescriptions when last surveyed between October 2025 and January 2026. Within our own website review, only around 26% of the 462 independent FOP websites we reviewed included any prescription price information, and only 38 of these FOPs included information on additional prescription fees during our review in January 2026.⁵⁵

Table 1.13: Additional prescription fees charged by independent FOPs, website reviews, 2026 (£)

	Number of sites	Min	Lower quartile	Median	Upper quartile	Max	Median as % of median primary fee
PCW [3<]	16	5.00	11.70	15.75	22.00	31.00	69%
CMA review*	38	8.00	10.52	17.78	28.11	28.11	89%

Note: *One large multi-site FOP has the same additional fee across its sites and accounts for 12 of the 38 FOPs in the analysis.
Source: CMA calculations based on [3<] response to RFI. and CMA website review, January-March 2026.

- 1.63 The median additional prescription fee charged by FOPs included in the PCW [3<] dataset was £15.075 compared to £17.78 in the sample of FOP websites reviewed by us. The median additional fee charged by FOPs in the PCW dataset was 69% of the median primary fee observed in the dataset. We observed a higher proportion in our review of FOP websites, with the median additional fee being 89% of the median primary fee.
- 1.64 As discussed above, all datasets that rely primarily on prices published by FOPs on their websites are likely to be affected by publication bias. Given the limited size of these datasets and the biases common to all website-based datasets, we

⁵⁴ All these businesses allowed multiple medicines to be issued on the same prescription, which could explain why they indicated that they do not charge for additional prescriptions. Another veterinary businesses, operating three sites, submitted that it did charge an additional prescription fee but allowed up to ten medicines to be issued on the same prescription – meaning the vast majority of pet owners would not pay an additional prescription fee. We note that another of these businesses, operating four sites, charged £20 for both primary and additional prescriptions in 2024 but lowered prices in 2026 to £16 for a primary prescription and nothing for additional prescriptions.

⁵⁵ Some FOPs explicitly stated additional prescription fees as ‘additional item’ fees or similar. We have also considered ‘per item’ and ‘per medicine’ fees to be both primary and additional fees.

do not consider that the additional prescription price information obtained from these sources are likely to be reliable.

Other prescription fees

- 1.65 Some FOPs within our datasets appear to offer other types of prescriptions not captured by the primary and additional fees described in the rest of this appendix. These fees, normally for repeat medications or charity work, represented a small proportion of the total number of prescriptions issued within our datasets and therefore have not materially influenced our analysis of the appropriate level for the prescription price control.
- 1.66 We note that some FOPs have different prices for repeat prescriptions, but this was not common practice. Only 13% of the [X] members of a buying group [X] that we surveyed said that they had a different price for repeat prescriptions.⁵⁶ In our website review we found that 1 out of the 91 practices that published a primary prescription fee, also published a separate fee for repeat prescriptions.⁵⁷ None of the 59 independent FOPs included in the dataset provided by a PCW [X] were recorded as offering a separate price for repeat prescriptions.⁵⁸ Other datasets did not specifically list separate prices for repeat prescriptions. We have therefore decided it is not appropriate to set a price cap for repeat prescriptions, and that we would not have the data to do so. We however note that 'repeat prescriptions' are one of the prices that FOPs would be required to publish (if relevant to them) as part of the price list remedy.
- 1.67 We found that many FOPs offered a small number of prescriptions at a low or zero price to charities or on a charitable basis.⁵⁹ However, we did not find any examples of these fees in datasets based on website reviews and therefore it appears that FOPs do not routinely advertise the availability of these prescriptions or use them as part of their competitive offering. As a result, we consider that FOPs that have chosen to offer these discounted prescriptions in the past will continue to do so in the future to support vulnerable pet owners and for pet welfare, regardless of the presence of a prescription price control.

⁵⁶ [X] of [X] veterinary businesses, representing 12% of sites (14 out of 113).

⁵⁷ Results of our website review, January 2026.

⁵⁸ Including LVG sites, 20 out of the total 397 sites in the dataset listed a price for repeat prescriptions. All of these sites were owned by Pets at Home. CMA calculations based on [X] response to RFI.

⁵⁹ As discussed above, we tested the impact of removing these fees from our analysis of LVG FOPs and found it made no difference to the median primary or additional prescription fees charged by any LVG.

2. Supporting evidence on other aspects of the prescription fee caps

Evidence on possible variations in the level of the prescription fee caps

- 2.1 As set out in part B section 5, Prescription price cap remedy, we considered whether the levels of the caps should be varied to account for known differences in the cost bases of veterinary businesses operating FOPs – in particular, variations in regional wages, which make up most of the costs of providing written prescriptions. Our analysis indicates that the levels of written prescription fees do not currently vary in line with regional differences in the way that we would expect to see if they were set based on regional cost differences.
- 2.2 We analysed primary prescription fees charged by 2,416 LVG FOPs in 2024.⁶⁰ We found that when comparing the median fees charged in each region, there were significant inconsistencies between LVGs with regards to the regions in which they charged more or less for a written prescription. For example, one LVG [X] had its lowest fees in London, while another LVG [X] had its highest fees in London, and others ranged in between.⁶¹ These results are shown in Table 2.1 below.
- 2.3 As outlined in Part B Section 5, Prescription price cap remedy, as an additional indicative sense check of whether any LVGs varied the cost of their primary prescriptions in line with what would be expected from regional differences in the cost of staff time, we compared each LVG's primary prescription fees against regional average gross weekly wages for a full-time employee in the UK.⁶² We found that, as shown in Table 2.1, the primary prescription fees at most LVGs did not vary in line with differences in the cost of labour between regions.⁶³

Table 2.1: Median primary prescription fees by LVG in 2024 compared to average gross weekly wages by region

Region	Gross weekly wage	[X]	[X]	[X]	[X]	CVS	[X]
Wales	669.80	-*	20.00	22.84	23.00	24.70	30.00
North East	707.91	N/A	20.00	28.08	24.00	24.70	29.40
East Midlands	724.20	18.50	20.00	35.00	22.00	24.70	28.98
Yorkshire and the Humber	749.26	20.50	21.00	27.00	24.00	24.70	31.00

⁶⁰ This analysis excludes independent FOPs as we were unable to obtain location information for a large majority of the independent FOPs for which we gathered prescription pricing data.

⁶¹ In this subsection, we are always referring to the median fee charged by an LVG in a given region.

⁶² [EARN05: Gross weekly earnings of full-time employees by region - Office for National Statistics](#). One limitation of this data series is that it measures the weekly wages of all workers and so is not specific to wages for veterinary staff (ONS data specifically related to veterinary wages does not include any measure of regional variation). Given we are using this data as an additional indicative sense check, we do not consider it necessary to use regional wage data specific to veterinary staff. We note that this wage data is also generally correlated with ONS data on private rents, which may give an indication of the relative cost of vet premises in each English region. [Private rent and house prices, UK - Office for National Statistics](#), figure 6.

⁶³ We also asked the LVGs about any central policies or guidance on prescription pricing. Some had no central policy, while others gave broad pricing guidance. None had any policy on regional price variation.

Scotland	760.81	21.16	20.00	27.30	23.00	24.70	30.49
South West	762.97	21.74	20.25	22.20	23.00	24.70	31.30
North West	764.18	22.50	20.00	25.61	22.00	24.70	29.50
West Midlands	792.99	22.84	20.25	22.50	24.00	24.70	30.00
South East	860.60	20.76	21.00	21.00	24.00	24.70	30.00
East of England	862.37	22.70	19.00	22.80	24.00	24.70	30.00
London	973.60	26.80	20.06	19.13†	24.00	24.70	32.86

*Figure removed as LVG [X] provided data on only one FOP in Wales.

† LVG [X] provided data on only two FOPs in London.

Source: CMA analysis of data provided by LVGs (in response to RFI 21, Q2). Data on average (mean) gross weekly wages was obtained from the ONS and reflects October to December 2024. [EARN05: Gross weekly earnings of full-time employees by region - Office for National Statistics](#)

Notes: Results for Northern Ireland and the Channel Islands are not reported as most LVGs do not operate a significant number of FOPs in these regions. We note that where this information was available for one LVG [X], it charged its lowest prescription fees in these regions. For each LVG, the highest and lowest median fees are in **bold** except for CVS where a flat fee is charged across all regions. LVG data covers more than 80% of sites for all LVGs except [X], which provided data on only 68% of sites in 2024. Columns are in ascending order of median fee in Wales.

2.4 For all but one of the LVGs, there was no clear relationship between regional prices and gross weekly wage data:

- (a) Most LVGs charged relatively similar fees across their FOPs in different regions. For three LVGs (CVS, [X], [X]), the difference in price between their highest and lowest priced region was £2 or less.⁶⁴ This was in strong contrast to a much wider dispersion in average gross weekly wages, with average wages in London around 45% higher than in Wales.
- (b) Only CVS has a fully centralised approach to prescription pricing, and it has a uniform fee across the UK.
- (c) Only one LVG [X] charged its highest median fee in London and lowest median fee in East Midlands.⁶⁵ In addition, its median fees in other regions were generally consistent with the ranking predicted by differences in average gross weekly wages and, excluding Wales due to data constraints, the magnitude of these differences correlates well with the differences we observe in the wage data.
- (d) Other LVGs appeared to take contradictory approaches, with regional pricing strategies which were neither consistent with the approach taken by the LVG described above [X], or with one another, and bore no clear relationship to the regional wage data. One LVG's [X] lowest fees were in the East Midlands,⁶⁶ the same region in which another LVG [X] charged its highest

⁶⁴ CVS had the same median fee in every region.

⁶⁵ In the submitted data, the LVG charged a lower fee in Wales. However, we note that this LVG provided data for only one FOP in Wales, so this may not be representative of the LVG's prescription fee pricing in Wales.

⁶⁶ This LVG [X] also charged this fee in the North West.

fee. Two LVGs [redacted] and [redacted] charged their highest median fees in London, whereas this was another LVG's [redacted] least expensive region.⁶⁷

- 2.5 As a further sense check, we also compared regional price differences using data on additional prescription fees, and repeated both analyses using 2023 data. Despite some differences in the relative level of some LVGs' regional fees compared to 2024,⁶⁸ the trends we observe in both primary and additional median prescription fees do not align with regional differences, and are not aligned to variations in the cost of staff time.
- 2.6 Therefore, we do not consider that there is any clear evidence that regional pricing variations significantly inform prescription price setting in the current market and, consequently, we judge that it is appropriate to set single UK-wide caps on prescription fees.

Evidence on future changes to the prescription fee caps

- 2.7 In Part B Section 5, Prescription price cap remedy, we set out that the level of the prescription fee price controls will vary over time by CPI inflation. In selecting an appropriate metric by which to increase the caps over time, we considered several factors. Our view is that a suitable inflation metric should:
- (a) ensure that the level of the fee caps continues to reflect broad-based changes in the costs faced by veterinary businesses over time;
 - (b) not be unduly altered by the implementation of our remedies package, meaning that the price of a written prescription and adjacent services should not constitute a significant part of the price index; and
 - (c) not be easily manipulatable by market participants.
- 2.8 As discussed in part B section 5, Prescription price cap remedy respondents to the PDR proposed alternative measures of inflation that they considered more suitable for the veterinary services market.
- 2.9 Respondents proposed the following alternatives, which we do not consider would be suitable.
- (a) **CPI for veterinary and other services for pets (vet CPI):**⁶⁹ IVC proposed adjusting the level of the fee cap by vet CPI, on the basis that it more

⁶⁷ However, we note this LVG's [redacted] median primary prescription fee in London is based on a sample of only two FOPs so may not be representative of the LVG's pricing in London.

⁶⁸ For example, in 2023 [redacted] highest primary prescription fee was in the North East, although this was its lowest priced region in 2024. CMA analysis of data provided by LVG [redacted] in response to RFI 8, Q1.

⁶⁹ [CPI ANNUAL RATE 09.3.5.0 Veterinary and other services for pets 2015=100 - Office for National Statistics](#).

accurately captures veterinary price inflation and is consistently higher than general CPI.⁷⁰ There are several reasons why we consider that this index would not be an appropriate measure by which to adjust the fee caps. First, the index is composed of only two items – annual booster injection fees and daily fees for dog kennelling. It is not clear that the prices for these services are closely related to or correlated with the cost base underpinning a written prescription fee. Second, any market participant with significant market share could influence the index by raising the price of one of these items. Finally, it is possible that the structurally higher inflation noted in the veterinary services sector may in part be due to the market not functioning well. As our package of remedies will take time to be implemented and strengthen competition, increasing the fee caps by vet CPI may entrench an unduly high cap level in this period.

- (b) **General increases in healthcare costs:** Medivet submitted that increases in total UK healthcare expenditure are a more appropriate comparator for veterinary services than general CPI.⁷¹ However, we consider that a total spend figure such as UK healthcare expenditure is not appropriate for measuring price inflation. This is because, in a scenario where prices remain constant, total spend could still rise as a result of increased consumption and the ageing population. In any case, as most UK citizens use the NHS, healthcare expenditure levels are influenced by political factors in a way that is not the case for the price of veterinary care.

- 2.10 Our assessment is, therefore, that CPI inflation is the most appropriate metric by which to vary the level of the prescription fee caps over time.

Evidence on costs related to written prescriptions

- 2.11 As discussed in Part B Section 5, Prescription price cap remedy, in considering the level at which to set caps on prescription fees, we explored a range of evidence relating to the costs of providing a written prescription including by seeking cost information from veterinary businesses.
- 2.12 We were, however, unable to obtain or calculate robust estimates for these costs because incremental costs associated with the sale of veterinary medicines are not monitored during the ordinary course of business by veterinary businesses.⁷²

⁷⁰ IVC response to the PDR, paragraph 5.1(iii).

⁷¹ We note that this submission was not specifically about the best metric to use to adjust the prescription fee caps. Medivet response to the PDR, paragraph 3(c).

⁷² As set out in part A, section 11: Veterinary medicines, sub-section FOPs charge a range of fees in relation to veterinary medicines. We focused our requests for this information on LVGs instead of independent FOPs because we expected the larger firms were more likely to hold this information, and they represent a significant share of the market. In addition, we invited submissions from all FOPs on the costs involved in producing a written prescription in our [Remedies Working Paper](#), p 157.

As noted below, FOPs vary in which cost lines they consider should be recovered through the prescription fee (and therefore cost estimates may not be comparable), and we would expect a lot of variation between different prescriptions, due to the variation in medicine complexity and administrative issues (such as pet owners' delay in confirming which online pharmacy they want to use). Therefore, the lack of any systematically collected or broadly comparable data makes it impossible to calculate a robust estimate of the true average cost of writing a prescription in all instances.

- 2.13 The subsections below set out the evidence we have gathered on the time and costs involved in writing a prescription, including some estimates of the level at which a cost-based prescription fee might be set. To the degree that we have any detailed information on time or costs of producing a prescription, these are for a single medicine. When there are multiple medicines prescribed within a single consultation, we do not have information on the specific time or costs attributable to these additional medicines. However, some respondents to our PDR submitted that at least some of the steps involved in producing a prescription are duplicated for each additional medicine.⁷³

Submissions estimating the professional time required to write a prescription

- 2.14 In response to the PDR, we received further cost information from respondents, which we have detailed below. However, these were not a suitable basis for us to be able to make robust estimates. Where we received more detailed analysis, in which FOPs had carried out specific time measurements of the process required for writing a prescription, these showed very large variations in time.
- 2.15 A buying group [X] provided us with a 'time in motion' study, in which 14 FOPs that were members of the buying group [X] tested the time taken to complete each of the administrative steps involved in creating two or three specific prescriptions.⁷⁴ Although the steps followed by each FOP in the study were not consistent as prescription writing processes vary, FOPs did not include the medical assessment of the animal or other clinical decision making in their time estimates.⁷⁵ This analysis found very wide variation, with total time estimates between 2 minutes 23 seconds, and over 16 minutes. The median time was 8 minutes 45 seconds.⁷⁶
- 2.16 The buying group [X] highlighted that there was 'a clear link between the time taken and the level of [PMS] integration with prescription production software',⁷⁷

⁷³ For example, responses to the PDR: Roker Park Vets, p 5, Priory Veterinary Group, p 2; Moore Scarrott, p 12, VetPartners, p 56, BVA BVNA BSAVA SPVS VMG response to the PDR, p 22.

⁷⁴ Buying group [X] submission, p.1.

⁷⁵ [X] response to putback, pp 2-3.

⁷⁶ Buying group [X] submission, p.1.

⁷⁷ Buying group [X] submission, p.1.

with three out of 11 PMS that were used by the FOPs in the study capable of direct emailing prescriptions, but only one FOP was able to fully utilise this function.⁷⁸ The buying group [X] submitted that practices which were more familiar with the production of written prescriptions had more efficient processes.⁷⁹ All of the FOPs which were part of the study chose to scan and email prescriptions to pet owners' chosen pharmacies, which the buying group [X] noted makes a significant difference to the total time taken to write a prescription.⁸⁰ The buying group [X] stated that 'Until now, the choice of [PMS] by a practice has not been influenced by the ability of the system to seamlessly and efficiently produce written prescriptions'.⁸¹

- 2.17 Moore Scarrott, an accountancy firm specialising in veterinary businesses, also stated that it carried out a 'time in motion' study. However, it provided no details on the number of FOPs or prescriptions covered, or whether there was any consistency of which tasks were included or what these might have been.⁸² Moore Scarrott submitted that the time taken to produce a written prescription 'normally ranges from 13 minutes to 20 minutes'.⁸³ It stated that the time required is influenced by the complexity of the case and a number of other factors, although it did not specify which other factors.⁸⁴
- 2.18 Some respondents to the PDR gave estimates of the time required to write a written prescription, which vary widely: six respondents estimated between 3 and 5 minutes, five estimated between 5 and 10, and ten estimated that it would take 10-15 minutes. These are, of course, individual assessments, and as a result may rely on differing and unclear methodologies. However, they corroborate the evidence above which indicates that the time taken to produce a written prescription varies significantly between FOPs.
- 2.19 As discussed below, most LVGs did not provide detailed estimates of the time or cost involved in producing a written prescription. Without providing detailed timing estimates, some LVGs submitted that it was important that their prescription fees accounted for the non-trivial amount of professional time required.⁸⁵ One LVG, which proactively submitted its estimate of the costs associated with issuing written prescriptions, estimated that the administrative actions involved in

⁷⁸ Buying group [X] submission, p.2.

⁷⁹ Buying group [X] submission, p.3.

⁸⁰ Buying group [X] submission, pp.1-2.

⁸¹ Buying group [X] submission, p.2.

⁸² We note that Moore Scarrott did provide a list of steps that may be involved in producing a written prescription, but it did not confirm in its submission that these were the steps followed by all participants in its study. Moore Scarrott response to the PDR, pp 12-13.

⁸³ Moore Scarrott response to the PDR, p 13.

⁸⁴ Moore Scarrott subsequently used these time estimates to calculate the 'cost' to the FOP of writing a prescription. It did this by using its estimate of the hourly charge out rate for a small animal first opinion vet in a consult room. It calculated that, assuming a charge out rate of £160 an hour, the cost of preparing a written prescription was between £41 and £64, including VAT. Moore Scarrott response to the PDR, p 13.

⁸⁵ For example, IVC response to the PDR, paragraph 5.4(i); CVS response to the PDR, p 8.

prescription writing (that is, excluding clinical considerations) could take more than 20 minutes (sometimes including the actions of multiple staff members).⁸⁶

- 2.20 Taken together, this evidence shows that there is poor data on the time taken to produce a written prescription, with very little consistency in the range of activities included and few examples where any data has been systematically collected. In any case, there is very large diversity in the time it takes, which the buying group discussed above [X] suggested may be due to the degree of integration of PMS, and the familiarity of the FOP with the process. This indicates that there is a wide range of efficiency, both in PMS and in FOP processes.

Submissions on the costs of writing a prescription

- 2.21 As noted above, the LVGs do not monitor the incremental costs associated with the sale of veterinary medicines, and were therefore unable to provide estimates of costs associated with prescriptions. For example, one LVG [X] stated, in response to our proposal that we collect this data, that:⁸⁷

[X]'s internal records do not have the cost breakdown necessary to provide an estimate of the direct and indirect incremental costs associated with dispensing, injecting, and prescribing ...and [X]'s accounting systems do not enable attribution of costs to specific clinical or administrative activities (e.g. administering an injection, issuing a prescription). Any attempts of such estimation would be extremely onerous... Further, any attempts of such exercise would rely on substantial assumptions, which will likely vary significantly across LVGs (reducing any comparability of any results across the vet sector)...

- 2.22 Despite this, one LVG [X] did provide us with a proactive submission on the costs of written prescriptions. It sent a survey to 29 FOPs, requesting time estimates of different activities related to written prescriptions, and converted these into cost estimates based on the member of staff involved.⁸⁸ This LVG calculated that the cost of producing a written prescription for non-complex medicines was [£15-20] per item (exclusive of VAT), including a contribution to overheads. The fee was made up of both clinical activities ([X]) and administrative costs [X].⁸⁹ We consider that the cost associated with clinical services (for example, providing information on the use, side effects or correct administration of medication and

⁸⁶ LVG [X] submission, paragraph 5.2. This LVG submitted that clinical considerations could take approximately a further 15 to 20 minutes, or longer if the medicines are complex. LVG [X] submission, May 2025, paragraph 4.1.

⁸⁷ LVG [X] draft RFI 19 response, Q1.

⁸⁸ LVG [X] submission.

⁸⁹ We note that the total cost of prescribing complex medicines was estimated to be £21.80 to £26.40. However, it was estimated no more than 10% of pets require a complex prescription over the lifetime of their pet. [X] proactive submission, May 2025, paragraphs 2.7 and 4.2.

writing clinical notes) could be allocated to either the cost of a consultation or the prescription fee. The administrative costs (writing the prescription and responding to queries from a third-party retailer) are directly related to the prescription, and are more consistent and substantially lower than the clinical costs. However, even when including clinical costs, the LVG's estimate of total costs are below the level of the primary prescription fee cap.

2.23 Many respondents to the PDR highlighted the costs, time or processes involved in producing a written prescription. For example:

- (a) IVC stated that 'IVC aims to set prescription fees at approximately [~~X~~] % of the price of a consultation, as it considers this price level is reflective of the amount of time, effort, risk, and cost involved.'⁹⁰
- (b) CVS stated that 'the proposed level [of the prescription fee cap in the PDR, £16,] is too low to account for the professional time and skill required of vets and the costs incurred by FOPs to issue prescriptions'.⁹¹
- (c) Many independent vets highlighted concerns about the proposed cap level covering the professional time of producing a written prescription.⁹²
- (d) FIVP highlighted issues with PMS integration for generating prescriptions, and said that independent FOPs had limited ability to switch PMS providers or pressure PMS providers to improve their processes during the implementation period that was proposed in the PDR.⁹³

FIVP said that: [the implementation period] fails to account for additional compliance and IT burdens on small practices. Changing PMS system is a major task for a practice and is not seamless. The aim should be for all PMS providers to be encouraged to develop a similar method of generating prescriptions, creating secure links with pharmacies (to mitigate fraud and minimise costs) and delivering upon the other administration tasks required from this review. The LVGs are in an advantageous position as they can apply pressure upon a single IT provider to make changes whereas this is not the case within the independent sector.⁹⁴

⁹⁰ IVC response to the PDR, paragraph 5.4(i).

⁹¹ CVS response to the PDR, p 8.

⁹² For example: responses to the PDR: Ardmory Veterinary Clinic, p 6, Northvet Veterinary Group, p 5, Armour Veterinary Group Ltd, p 3.

⁹³ In the PDR, we proposed a six month implementation period for smaller FOPs.

⁹⁴ FIVP response to the PDR, p 21.

- 2.24 This highlights that PMS generating prescriptions or linking directly to online pharmacies are not commonplace, leading to manual and inefficient processes at most FOPs.
- 2.25 The evidence, including the submissions discussed above, also indicates that the costs included in the prescription fee vary between FOPs, and many FOPs use prescription fees to cover more than the direct cost of providing a written prescription to pet owners or an online pharmacy (either on paper or electronically). Activities that some veterinary businesses cover through prescription fees may be included as part of the consultation fee by others.⁹⁵ For example, explaining how to administer the prescribed medication to the pet owner and the potential side effects of the medication could be part of the dispensing and prescription fees or the consultation fee. Prescription fees may also cover the cost of anticipated follow-on activities (including answering queries from pet owners or third-party retailers), responding to any side-effects experienced by a pet, and on-going liability for the use of medication to treat the pet under care.⁹⁶ Some respondents to the Remedies Working Paper⁹⁷ and PDR⁹⁸ told us that the costs of measures to minimise errors or fraud are also included in the prescription fee by some FOPs.
- 2.26 Therefore, we consider that, given the data available and the variety of practices across the market, setting the level of prescription price controls based on any bottom-up cost analysis would not be reliable or robust. Our view is set out in more detail in part B section 5, Prescription price cap remedy.

Evidence on the relationship between primary fees and additional fees

- 2.27 We carried out analysis of the relationship between primary fees and additional fees, using the datasets that were drawn directly from PMS (the LVGs and data from one of the veterinary advisory businesses [3<]). We analysed the additional fee as a proportion of the primary fee, to understand how FOPs set prices. Where FOPs charged an additional prescription fee above £0, the most common approach was to charge between 50% and 60% of the primary fee, with 44% of these FOPs setting their prices in this range. Overall, 67% of FOPs set the additional medicine at under 60% of the primary prescription price.

⁹⁵ As set out in part A, section 11: Veterinary medicines, sub-section FOPs charge a range of fees in relation to veterinary medicines.

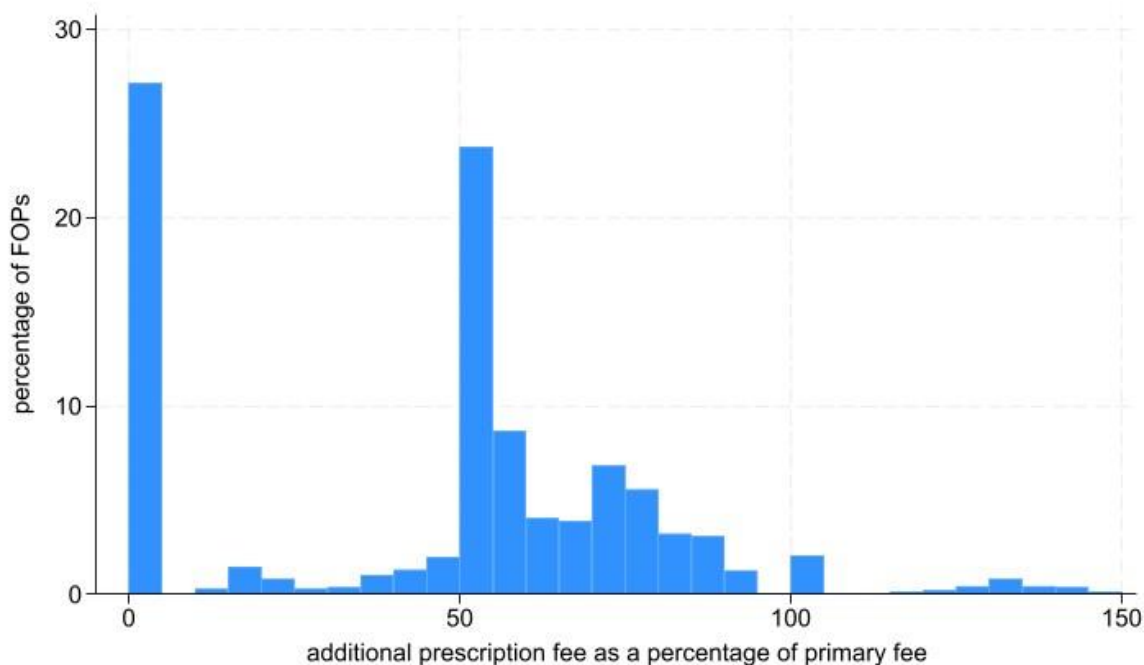
⁹⁶ As set out in part A, section 11: Veterinary medicines, sub-section FOPs charge a range of fees in relation to veterinary medicines.

⁹⁷ CMA summary of RWP responses. For example, Respondent 7, Respondent 8, Respondent 23, The George Vet Group.

⁹⁸ Issues around prescription fraud and responses we received to the PDR are discussed in the part B section 5, Effects of the medicine remedies.

- 2.28 We analysed fees charged for primary and additional prescriptions by FOPs that had data for both fees in 2024 using the data submitted by five LVGs for which we could identify additional prescription fees, and data for independent FOPs provided by the one of the veterinary advisory businesses [X]. As described above, we calculated a single average primary and additional fee for each site, where the LVG averages are weighted by the quantity of prescriptions sold at each site. This resulted in a dataset containing 1,320 LVG FOPs and 52 independent FOPs.
- 2.29 We assume that for CVS, the additional prescription fee is £0.00 as the primary prescription fee covers the cost of up to five medicines for a single condition.⁹⁹ Hence, all but one of the 27% of FOPs that set the additional prescription fee at 0% to 5% of the primary prescription fee are CVS FOPs. Only a small proportion of FOPs set their additional fee higher or equal to their primary fee – 3% of FOPs set the additional fee at over 100% of the primary fee and 2% of FOPs set their additional fee at the same level as their primary fee.

Figure 2.1: distribution of additional prescription fees as a percentage of primary prescription fees in 2024 for five LVGs and independent FOPs with available data

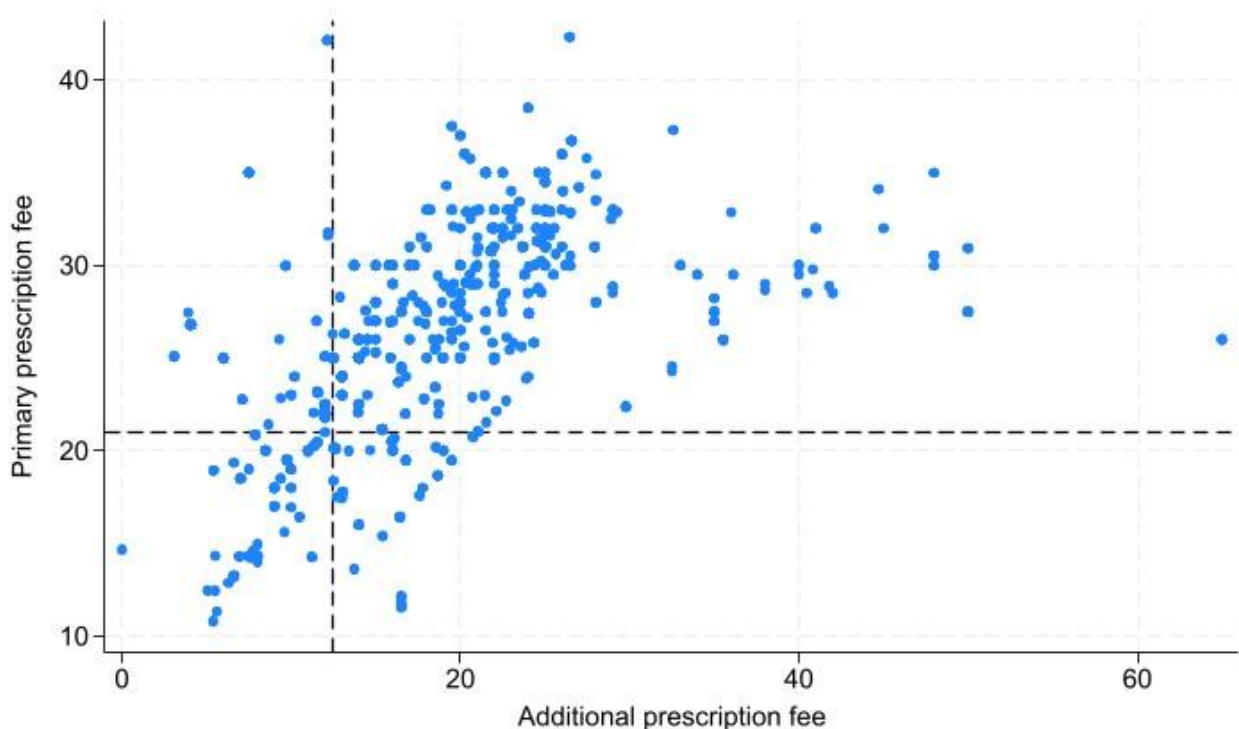


Source: CMA analysis of data provided by LVG responses to RFI 21 Q2 and veterinary advisory business [X] response to RFI 2 .Note: The figure excludes 13 FOPs where the additional prescription fee was over 150% of the primary prescription fee as these were likely data errors.

⁹⁹ CVS further submitted that, in practice, it is common for its vets to charge a single prescription fee for multiple medicines even when these are for different conditions. CVS response to RFI 21, Q1.

- 2.30 As set out in Part B Section 5, Prescription price cap remedy, we tested whether FOPs that set lower primary prescription fees tended to price higher for additional medicines, and vice versa. Our analysis indicates that FOPs with lower-priced primary prescriptions tend to have lower-priced additional prescriptions, and FOPs with higher-priced primary prescriptions tend to have higher-priced additional prescriptions.
- 2.31 We analysed fees charged for primary and additional prescriptions by FOPs that had data for both fees in 2024 using the data submitted by four LVGs for which we could identify additional prescription fees, and data for independent FOPs provided by the one of the veterinary advisory businesses [X]. As described above, we calculated a single average primary and additional fee for each site, where the LVG averages are weighted by the quantity of prescriptions sold at each site. This resulted in a dataset containing 952 LVG FOPs and 52 independent FOPs.
- 2.32 As shown in Figure 2.2 below, primary and additional prescription fees are strongly positively correlated, with only a small minority of FOPs charging high additional fees relative to primary prescription fees or vice versa.

Figure 2.2: Primary and additional prescription fees in 2024 for four LVGs and independent FOPs with available data



Source: CMA analysis of data provided by LVG responses to RFI 21, Q2 and veterinary advisory business [X] response to RFI 2.

Note: Vertical dashed line at additional prescription fee cap (£12.50) and horizontal dashed line at primary prescription fee cap (£21). Although not included in the figure, we note that CVS has an additional prescription fee of £0.00 as the primary prescription fee covers up to five medicines for a single condition.¹⁰⁰

2.33 We repeated this analysis using 2023 data and again found a strong positive correlation between primary and additional prescription fees. Therefore, we consider that there is no evidence that FOPs that set a lower primary prescription fee tended to price higher for additional medicines or vice versa. Our findings indicate that FOPs may set the level of their additional prescription fee relative to the level of the primary fee, without a clear pricing rationale independent of the primary fee.

¹⁰⁰ CVS further submitted that, in practice, it is common for its vets to charge a single prescription fee for multiple medicines even when these are for different conditions. CVS response to RFI 21, Q1.